Tropical Infections

CHAPTER 4.12	TROPICAL INFECTIONS		
Infections and Likely Organisms	**Suggested Treatment**		**Notes**
	Recommended	**Alternative**	
Malaria			
Plasmodium Falciparum **Uncomplicated**	Artemether / Lumefantrine (Riamet®) *1 tablet = Artemether 20 mg / Lumefantrine 120 mg* **5 - 14 kg**: **Day 1:** 1 tablet single dose then 1 tablet again after 8 hours **Day 2 & 3:** 1 tablet q12H **15 - 24 kg:** **Day 1:** 2 tablets single dose then 2 tablets again after 8 hours		• Add Primaquine 0.75 mg/kg single dose q24H if gametocyte is present at any time during treatment. • Check G6PD status before giving Primaquine. • Both Artesunate/Mefloquine (ASMQ) and Riamet® are Artemisinin based Combination Treatment (ACT).

Infections and Likely Organisms	Suggested Treatment		Notes
	Recommended	Alternative	
	Day 2 & 3: 2 tablets q12H **25 - 35 kg:** **Day 1:** 3 tablets single dose then 3 tablets again after 8 hours **Day 2 & 3:** 3 tablets q12H **> 35 kg:** **Day 1:** 4 tablets single dose then 4 tablets again after 8 hours **Day 2 & 3:** 4 tablets q12H		
Plasmodium Falciparum **Severe**	Artesunate IV **<20 kg:** 3 mg/kg/dose at time of 0H, 12H, 24H and then once daily **≥20 kg:**		• IV Artesunate should be given for a minimum of 24 hours (3 doses) or until the patient is able to tolerate orally and after that to complete treatment with a complete course of oral ACT (3 days of ASMQ or Riamet®).

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	2.4 mg/kg/dose at time of 0H, 12H, 24H and then once daily		
Plasmodium Vivax, Plasmodium Knowlesi, Plasmodium Malariae	Artemether / Lumefantrine (Riamet®) *Follow the dose as per Plasmodium falciparum* **PLUS** Primaquine 0.5mg/kg/ dose (max 30 mg) PO q24H for 14 days		• Severe *Plasmodium Vivax, Knowlesi* or *Malariae* should be treated as severe Plasmodium Falciparum. • Chloroquine-resistant *Plasmodium Vivax, Knowlesi* or *Malariae* - please consult ID specialist.
Mixed Malaria Infection	Treat as *Plasmodium falciparum*		
Congenital Malaria	**Total Chloroquine 25 mg base/kg orally divided over 3 days** Day 1: 10 mg base/kg PO STAT then 5 mg base/kg PO 6 hours later		

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	Day 2 & 3: 5 mg base/kg PO q24H		
Malaria Chemoprophylaxis	Please refer to **Chapter 3.18: Non-surgical Prophylaxis**.		
Tuberculosis *Mycobacterium tuberculosis*	Isoniazid (H) 7 - 15 mg/kg/day PO (max 300 mg) + Rifampicin (R) 10 - 20 mg/kg/day PO (max 600 mg) + Pyrazinamide (Z) 20 - 40 mg/kg/day PO (max 2000 mg) ± Ethambutol (E) 15 - 25 mg/kg/day PO (max 1000 mg) ± Streptomycin (S) 20 - 40 mg/kg/day IM (max 1000 mg)	2nd-line treatment / resistant mycobacterium TB – please consult ID specialist	• Decision for duration of treatment depends on whether pulmonary or extrapulmonary TB. • Intensive phase requires at least three drugs and maintenance two drugs regime.

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Melioidosis *Burkholderia pseudomallei*	**Induction** Ceftazidime 50-75 mg/kg/dose IV q8H for at least 14 days* **Maintenance** Co-amoxiclav 20 mg/kg/dose (Amoxicillin component) PO q8H for total 12-20 weeks **OR** Co-trimoxazole (Trimethoprim component) 4 mg/kg/dose PO q12H for 12-20 weeks	**Induction** Meropenem 20 - 30 mg/kg q8H for at least 14 days (may be considered in life threatening case)	• Localised melioidosis abscess should be drained. • *Deep seated abscess: duration of induction is 4-8 weeks. • **Maintenance (if high risk of relapse):** **> 8 years old:** Co-trimoxazole (Trimethoprim component) 4 mg/kg PO q12H for 12 - 24 weeks + Doxycycline 2 mg/kg PO q12H for total 20 weeks
Leptospirosis *L. icterohaemorrhagiae* *L. canicola*	**Mild:** Amoxicillin 25-30 mg/kg/dose PO q8H for 7 days **Severe:**	Ceftriaxone 50 mg/kg IV q12H for 7 days **>8 years old:** Doxycycline 2 mg/kg/dose PO q12H for 7 days	• Penicillin is the drug of choice for moderate to severe disease.

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	Benzylpenicillin 100,000 units/kg IV q6H for 7 days		
Scrub Typhus *Rickettsia tsutsugamushi*	Doxycycline 2 mg/kg/ dose PO q12H for 5-7 days	**If resistance to Doxycycline is suspected:** Azithromycin 10 mg/kg/ dose PO q24H for 5 days	